

Credentialing and Supervision of Contract Staff

Verifying Qualifications and Maintaining Oversight

Regulation: 42 CFR §482.12(a) | §482.22

POLICY OVERVIEW

Hospitals bear ultimate responsibility for the qualifications and performance of all individuals providing patient care, regardless of employment status. Robust credentialing and privileging processes ensure that only qualified practitioners provide care within their demonstrated competencies.

KEY REQUIREMENTS

1 Initial Credentialing

All practitioners must complete a rigorous initial credentialing process including: verification of education and training, licensure verification with primary source, board certification, work history, professional references, malpractice history review, and criminal background check.

2 Privileging Process

Clinical privileges must be specific to each practitioner's demonstrated competencies and within the scope of the hospital's services. Privileges must be based on education, training, experience, and demonstrated current competence.

3 Ongoing Professional Practice Evaluation (OPPE)

Ongoing monitoring of practitioner performance must occur at least every 12 months. OPPE data must include: case volume, outcomes, quality metrics, and peer review findings. Results must inform privilege renewal decisions.

4 Focused Professional Practice Evaluation (FPPE)

Focused evaluation must be triggered by: initial privilege grant, new privileges, identified performance concerns, or significant adverse events. FPPE criteria and monitoring methods must be defined in advance.

5 Contract Staff Requirements

Contract and locum tenens practitioners must meet the same credentialing and privileging standards as employed staff. Credentials may not be accepted from a staffing agency without independent primary source verification.

COMPLIANCE NOTES

The Medical Staff Bylaws govern the credentialing and privileging process. The Credentials Committee and Medical Executive Committee have oversight. The Governing Body makes final privileging decisions. Inadequate credentialing is a serious compliance risk.

COMMON SURVEY CITATIONS

Cited under §482.12(a)(1) – Governing body responsibilities for medical staff, §482.22(a) – Medical staff organization, and §482.22(a)(1) – Credentialing requirements.